

EVIDENCE LOG

For Neck Pain (Cervical Strain) VA Disability Claims

THE SMOKING GUN EVIDENCE

After years of navigating the VA claims process—as veterans ourselves—we discovered something that changes everything: **the single most powerful piece of evidence you can submit isn't officially part of a Fully Developed Claim.**

"No more convincing. Just corroboration."

When you walk into a C&P exam without this documentation, the examiner's job is to **investigate whether your condition exists and how severe it is.** They're playing detective. You're trying to convince them.

When you walk in WITH a properly completed Evidence Log, the dynamic completely shifts. The examiner's job becomes **corroborating what you've already documented.** They're not investigating—they're verifying. That's a fundamentally different exam.

HOW VA RATERS ACTUALLY EVALUATE YOUR CLAIM

Every VA disability rating—regardless of condition—is based on three core factors. Raters are trained to look for evidence of these specific elements:

FACTOR	WHAT IT MEANS	WHAT RATERS LOOK FOR
FREQUENCY	How often do your symptoms occur?	Daily? Weekly? Constant? with specific triggers?
SEVERITY	How intense are your symptoms when they occur?	Pain levels (1-10)? Range of Motion loss? Duration?
FUNCTIONAL IMPACT	How do symptoms affect your daily life?	Can't work? Can't sleep? Can't lift kids? Missed events?

Why This Evidence Log Works: The template you're about to use is specifically designed to capture all three factors in a format that's easy for raters to process. When a rater opens your file and sees 30 days of documented symptoms with frequency, severity, and functional impact clearly recorded, they don't have to guess or interpret. The evidence speaks for itself.

DO NOT FILE ANY CLAIM UNTIL THIS LOG IS COMPLETED AND INCLUDED WITH YOUR SUBMISSION

NECK CONDITIONS: CONDITION-SPECIFIC GUIDANCE

The VA rates the neck (Cervical Spine) based on **Range of Motion (ROM)** and **Incapacitating Episodes**.

What to Log:

- **Stiffness:** Can you look over your shoulder to change lanes?
- **Radiculopathy:** Numbness/tingling radiating down into arms/fingers? (Log this carefully!)
- **Spasms:** Muscle tightness causing head tilt?

Rating Criteria:

RATING	LIMITATION
10%	Pain/Tenderness or mild limitation of motion.
20%	Forward flexion limited to 30° or combined ROM limited.
30%	Forward flexion limited to 15°.

COMPLETED EXAMPLE: 7-DAY EVIDENCE LOG

Below is an example of a properly completed Evidence Log.

DATE	TIME	SYMPTOMS & SEVERITY	FUNCTIONAL IMPACT
Mon 1/6	1700	Neck locked up after computer work. Spasms. Pain 7/10.	Had to lie flat on floor. Could not drive home—wife picked me up.
Tue 1/7	All day	Tingling in right thumb/index finger (Radiculopathy).	Dropped pen twice. Difficulty typing.

YOUR EVIDENCE LOG: 30-DAY TEMPLATE

Instructions: Complete one row per day for at least 30 consecutive days.

VETERAN NAME: _____ **CLAIM #:** _____
_____ **CONDITION:** Neck Pain (Cervical Strain) (DC 5237)
START DATE: _____

DATE	TIME	SYMPTOMS & SEVERITY	FUNCTIONAL IMPACT
Day 1			
Day 2			
Day 3			
...			
Day 30			

Note: Print this page multiple times to cover 30 days.